Can the spread of COVID-19 be modelled by a reaction-diffusion equation?

Oliver Clift[1], James Emberton[1], Isaac Millen[1], Nicholas Mitchiner[1], Edward Phillips[1] and Dominik Szablonski[1]

[1]Department of Physics, University of Manchester

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Abstract

This study modelled the spread of disease in Manchester, using a reaction-diffusion equation, comparing this to COVID-19 data in Boston. The main assumptions of the model were a constant population density, a recovery time of 14 days, and negligible effects on immunocompromised individuals. The Euler method was used to solve the equation numerically. Our model successfully recreated spatial disease spread when compared to COVID-19 data in Boston, although it diverged from the data after ~30 days. Model extensions could involve including a travelling population, multiple outbreak centres and behavioural changes.

Introduction

Modelling disease spread is crucial for governments to develop effective contingency plans for large-scale pandemics. The UK's "inadequate response" to COVID-19 serves as evidence for poor governmental action [1]. Motivated by the need for better predictive tools, we attempted to model disease spread within Greater Manchester using a diffusion equation, chosen because the random large-scale motion of people is analogous to Brownian diffusion.

However, due to mass conservation, a standard diffusion equation is incapable of accurately modelling disease spread, because diseases constantly generate new infections. A reaction term is therefore added to account for both new infections and patient recovery, yielding the SIR-based framework described below.

Theory - Equations and Assumptions

Reaction-diffusion equations were originally developed to model chemical reactions involving microscopic particles in a fluid undergoing random, Brownian motion. The reaction-diffusion equation is given by,

$$\frac{\partial u}{\partial t} = D\nabla^2 u + R, \tag{1}$$

where $u = u(x, y, t)$ is some unknown function, D is a matrix of diffusion coefficients, modelling the net movement of particles from an area of high concentration to an area of low concentration, and $R = R(u, x, y, t)$ is the reaction term, which models how particles change from one type to another [2]. This equation is ideal for modelling disease spread since, on a large scale, the motion of people can be assumed to be random.

In this model, we define,

$$u(x, y, t) = S(x, y, t) + I(x, y, t) + R(x, y, t), \tag{2}$$

as the total population, which must be conserved. We have defined the following additional fields:

- $S(x, y, t)$ is the susceptible population (those who can catch the disease).
- $I(x, y, t)$ is the infected population (those who have the disease and can spread it).
- $R(x, y, t)$ is the recovered population.

We then define a reaction term consisting of two parts, the rate of new infection,

$$\frac{\partial S}{\partial t} = -\beta_0 \rho(x, y) SI, \tag{3}$$

and the rate of recovery,

$$\frac{\partial R}{\partial t} = \gamma I, \tag{4}$$

where β is the rate of infection, $\rho(x, y)$ is the population density field, and

$$\gamma = \frac{1}{\tau}, \tag{5}$$

where τ is the recovery time. This allows us to obtain the final differential equation,

$$\frac{\partial I}{\partial t} = D\nabla^2 u(x, y, t) + \beta_0 \rho(x, y) SI - \gamma I. \tag{6}$$

We chose to model COVID-19, since a large amount of data on its spread is available to compare the model to, as well as its high rate of infection.

Theory - Numerical Methods

The system of equations was discretised using the method of lines and a five-point stencil, meaning each spatial cell has four nearest-neighbours that contribute to its spatial gradient, as below,

$$\nabla^2 I \approx I_{i+1,j} + I_{i-1,j} + I_{i,j+1} + I_{i,j-1} - 4I_{i,j}. \tag{7}$$

The population density of each borough within Greater Manchester was modelled using a series of Gaussian functions centred on their approximate positions relative to each other using 2021 Census data. The population density is denoted in dimensions of people per area. Due to the discretisation of our model, we normalised the population densities.

The Euler method was used to solve equation (6), calculating the total S, I, and R fields. The Euler method is a first-order numerical procedure to for solving ordinary differential equations [3].

Results: Model Figure

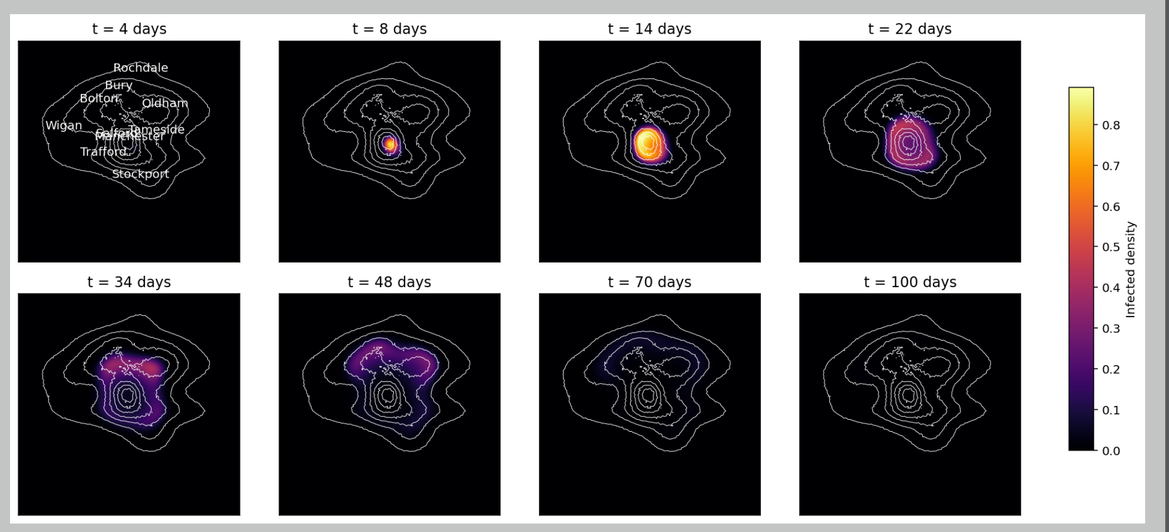

Figure 1: COVID-19 infection over time in Greater Manchester up to 100 days. Using parameters of initial transmission rate $\beta_0 = 0.48$ per day and diffusion coefficient of $D = 0.5$ m^2 s^{-1} and a mean recovery time of $\tau = 7$ days.

Results: Simulation vs Data

Infection of COVID-19 was modelled in Greater Manchester with an initial transmissibility of $\beta_0 = 0.48$ per day taking into account weather effects [4] while using a diffusion coefficient of $D = 0.85$ m^2 s^{-1}, and a mean recovery time of $\tau = 7$ days [5] and using the spatial density given for each county of Greater Manchester from the March 2021 Census data [6]. The model was compared to the COVID-19 infection map of Boston [7] as it is a city of comparable size thus allowing us to clearly compare the accuracy of our model. As seen our model is comparable to the Boston infection map up to the first 30 days. At later times, the model diverges from real data. While our model predicts rapid decrease in the infected population, whereas in real outbreaks, as in figure 2, the infection remains for significantly longer when compared to our simulation.

Results: Spread of Covid From Data

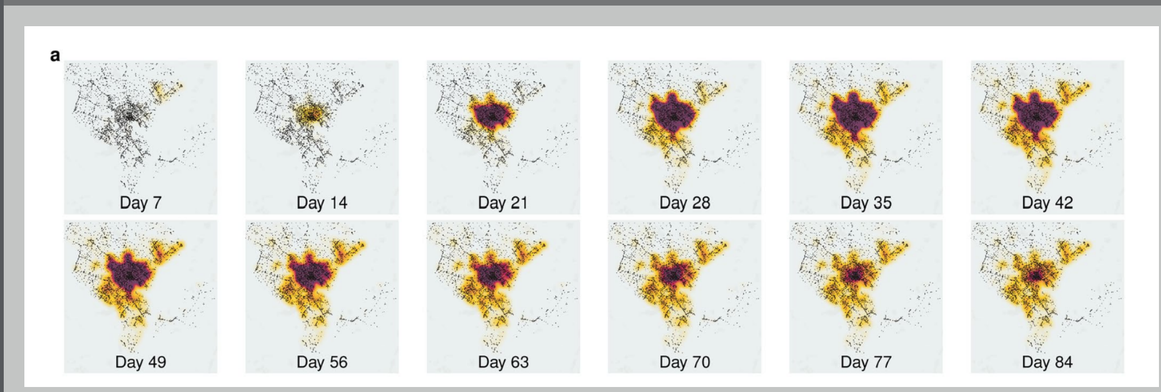

Figure 2: COVID-19 infection "Heatmap of infected (exposed and infectious) individuals every seven days: (a) Auto Sprawl;"[7] of Boston over 84 days. Figure 5 from " Activity-based epidemic propagation and contact network scaling in auto-dependent metropolitan areas" by Kumar, Nishant & Oke, Jimi & Nahmias Biran, Bat-hen,2021, Scientific Reports, Volume 11, p. 9. Copyright 2021 by Authors. CC BY "

Discussion

We found that our model was successful at modelling the initial spread of the COVID-19 pandemic. In particular, the initial growth seen in Fig.1 matches near perfectly data seen in Fig. 2. However, after this, the model starts to diverge from real life data. This is likely due to our assumption that people wouldn't isolate. In reality, as time went on, and more was understood about Covid, people started to self isolate when they first experienced symptoms, and the first lockdowns started to occur in March 2020 [8]. However, this doesn't make the idea of using reaction-diffusion useless or invalid in all cases. Instead, it implies that the best use of such a model is to predict what would happen should the spread of the virus not be contained for the initial spread.

Our model also fails to account for individuals travelling long distances, instead assuming that everyone remains in approximately the same location. In reality, this isn't true for multiple reasons. First, as a big city, people commute into central Manchester from surrounding boroughs and towns to shop and work. These people are likely to get infected in Manchester and travel back to their homes, causing the pandemic to spread to outer regions sooner than in the model. Greater Manchester isn't closed either with people travelling national and internationally every. Whilst an infected person leaving Manchester wouldn't change much, at least outside of the very early days, an infected person entering Manchester could lead to a new infection centre. In reality, it turns out that, "connectivity matters more than density in the spread of COVID-19" [9].

A reaction-diffusion model also fails to account for the virus, "Sticking around", as seen in figure 2. In our model, after around 30 days the area where our "patient zero" first appeared had been near completely cleared out as everyone in the area had caught the illness and recovered. This contrasts with the data seen above. In reality, not everyone in an area gets ill immediately, so the illness will stay around for longer rather than just spread to other areas. As such, even months on the infected density for the virus doesn't fall off close to 0 unlike our model. Despite this, the model may still be valid. Historically pandemics tend to slow down as they burn through all people in an area as herd immunity protects the population. As real life lockdowns (which aren't taken into account by our model) slowed down the spread of infection, it took far longer for herd immunity to be achieved in real life.

Conclusions

In conclusion, the reaction-diffusion equation successfully captured the initial spread of COVID-19, but diverged from real-world data at later times. This is due to simplifying assumptions, which a more sophisticated model, taking into account travelling population, multiple outbreak centres and behavioural changes would account for.

References

[1] Rhiannon Frowde, Edward S. Dove, and Graeme T. Laurie.
Fail to prepare and you prepare to fail: The human rights consequences of the uk government's inaction during the covid-19 pandemic.
Asian Bioethics Review, 12(4):459–480, 2020.

[2] Alan M. Turing.
The chemical basis of morphogenesis.
Philosophical Transactions of the Royal Society of London. Series B, 237(641):37–72, 1952.

[3] Leonhard Euler.
Institutiones Calculi Integralis, volume 1.
Academiae Imperialis Scientiarum Petropolitanae, Saint Petersburg, 1768.

[4] Talib Dbouk and Dimitris Drikakis.
Fluid dynamics and epidemiology: Seasonality and transmission dynamics.
Physics of Fluids, 33, 2 2021.

[5] National Health Service.
How to avoid catching and spreading COVID-19 infection, 2023.
Page last reviewed: 21 March 2023. Next review due: 21 March 2026.

[6] Office for National Statistics.
Census-based statistics UK: 2021.

[7] Nishant Kumar, Jimi Oke, and Bat-hen Nahmias Biran.
Activity-based epidemic propagation and contact network scaling in auto-dependent metropolitan areas.
Scientific Reports, 11, 11 2021.

[8] Institute for Government.
Timeline of UK Coronavirus Lockdowns, March 2020 to March 2021, 2021.

[9] Shima Hamidi, Reid Ewing, and Sadegh Sabouri.
Longitudinal analyses of the relationship between development density and the COVID-19 morbidity and mortality rates: Early evidence from 1,165 metropolitan counties in the United States.
Health & Place, 64:102378, 2020.